coursera

Depressive and Bipolar Disorders ✕

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Opening to Depressive and Bipolar Disorders

Lesson 1: Culture and Mood Disorders
✓ Focus of the Course
Reading • 2 min
✓ Classifying Psychopathology
Reading • 4 min
✓ Culture and Psychopathology
Reading • 4 min
✓ Culture and Intersectionality
Reading • 4 min
✓ Culture and Stigma
Reading • 4 min
✓ Check Your Understanding: Culture and Mood Disorders
Practice Assignment • Grade: 100%

Lesson 2: Introduction to Depressive and Bipolar Disorders
✓ Depressive and Bipolar Disorders
Video • 6 min
✓ Living with a Mood Disorder: A Case Study
Reading • 10 min
✓ Overview of Depressive and Bipolar Disorders
Reading • 10 min
✓ Check Your Understanding: Introduction to Depressive and Bipolar Disorders
Practice Assignment • Grade: 80%

Key Takeaways
✓ Key Takeaways: Opening to Depressive and Bipolar Disorders
Reading • 10 min

Module 2 Quiz
✓ Module Quiz: Opening to Depressive and Bipolar Disorders
Graded Assignment • Grade: 87.5%

Module 3
Depressive and Bipolar Disorders

Lesson 3: Overview of Depressive Disorders
✓ Depression: A Case Study
Reading • 5 min
✓ What Does Depression Feel Like?
Reading • 7 min
✓ Diagnostic Criteria and Prevalence for Depressive Disorders
Reading • 5 min
○ Check Your Understanding: Overview of Depressive Disorders
Practice Assignment • 12 min

Lesson 4: Overview of Bipolar Disorders
✓ Bipolar: A Case Study
Reading • 5 min
○ **What Does Bipolar Feel Like?**
Reading • 7 min
○ Diagnostic Criteria and Prevalence for Bipolar Disorders
Reading • 5 min
○ Check Your Understanding: Overview of Bipolar Disorders
Practice Assignment • 13 min

Key Takeaways
○ Key Takeaways: Depressive and Bipolar Disorders
Reading • 10 min

Module 3 Quiz
○ Module Quiz: Depressive and Bipolar Disorders
Graded Assignment • 16 min

Module 4
Biological Contributing Factors

Module 5
Psychological Contributing Factors

Module 6
Treatments for Depressive and Bipolar Disorders

Module 7
Suicide

Module 8
Conclusion

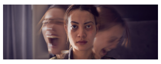
Module 9
Cumulative Quiz

Module 10
Course Resources

Module 11
PsycLearn Essentials APA Student Resources

What Does Bipolar Feel Like?

3:27 / 10:23 1x



Common Symptoms of Bipolar Disorders

Mood dysregulation, or difficulties in managing emotions, is the defining characteristic of bipolar disorders. In one state of bipolar disorders, people's moods become so elevated that their self-perceptions become distorted and their impulse control is impaired. In other states of bipolar disorders, people experience sadness and anhedonia similar to what people with severe depression endure. In each of these states, the intensity of people's moods is beyond what is typically seen in non-disordered populations.

Bipolar disorders are a group of mood disorders in which there is an alternation between episodes of depression and episodes of mania or hypomania with episodes of **euthymia,** a mania- and depression-free state of well-being and tranquility. Within the *ICD-11* diagnostic manual, bipolar disorders are identified by the type of manic episode (i.e., manic, mixed, or hypomanic), the severity of the symptoms (i.e., mild, moderate, or severe), and with or without psychotic symptoms.

Mania is often described as periods of elated moods in which people need little sleep, have seemingly boundless energy, and are hyper-focused on achieving their goals. Just given that description, mania sounds like a state that people might enjoy having. Many people with bipolar disorder feel that the manic episodes are when they are who they "really are." Although there are some pleasurable components of mania, being in a manic state is not an exclusively positive experience. First, not all people experience mania as an elevated mood. Some experience it as closer to agitation or irritability. Second, the energy that comes with mania can feel like a blessing, but also a curse. Some people feel like they're constantly being pressured to do something whether it's working on a project, pursuing a romantic encounter, or expressing whatever thoughts are racing through their heads. Finally, mania can lead to dangerous and destructive behaviors such as excessive, impulsive spending, pursuing foolish business ventures, or reckless and potentially dangerous sexual activity.

Common symptoms of mania include:

1. **Impulsivity** – A single-minded pursuit of whatever goal is currently important to someone in a manic state, regardless of the consequences to themselves or others (Santana et al., 2022). This can lead to risky behaviors such as dangerous driving, drug use, reckless spending, and sexual promiscuity.
2. **Financial mismanagement** – As a result of their impulsivity, people in a manic state will often make purchases that are out of line with their financial means (Richardson et al., 2018).
3. **Hypersexuality** – An increased libido causing a large increase in sexual behavior is a common symptom that, when combined with bipolar's impulsivity, often leads to risky sexual behavior (Staali, 2016), higher rates of sexually transmitted infections (Chen et al., 2018), and higher rates of unplanned pregnancies (Heffner et al., 2012; Kopeykina et al., 2016).
4. **Cognitive impairments** – People experiencing a manic episode often have racing thoughts that can be very distracting. This can lead to cognitive impairments such as significant problems with memory and the ability to focus attention (Simonsen et al., 2008).
5. **Pressured speech** – People in manic states experience their thoughts as so important and so fast-moving that they feel compelled to express them as quickly as they can (Weiner et al., 2019). Others will (correctly) perceive the person as being unable to stop talking about anything that crosses their mind.

Hypomania, a milder form of mania, is defined as a persistent mood state lasting at least several days characterized by mild elevation of mood or increased irritability and increased activity or a subjective experience of increased energy, accompanied by the other characteristic symptoms of mania. The symptoms of hypomania are not severe enough to cause marked impairment in occupational and social realms or in relationships; they do not necessitate hospitalization; and they do not include psychotic features such as delusions and hallucinations.

The depressive episodes experienced by people with bipolar disorder are very similar to those experienced by people with one of the disorders on the depressive spectrum. Attributes of depression in bipolar patients may include extremely low mood, low energy, and anhedonia. The contrast between this depressive state and a manic state is stark and may contribute to the high rate of suicide attempts among bipolar patients. Studies indicate that, at a minimum, between a quarter and a third of bipolar patients who cycle between mania and depression will attempt suicide, though some estimates suggest the number may be much higher (Carvalho & Vieta, 2017; Novick et al., 2010).

Sorrowing Old Man (At Eternity's Gate)

Note. Vincent van Gogh is believed to have suffered from bipolar disorder along with other psychological disorders. This image, titled *Sorrowing Old Man (At Eternity's Gate)*, is believed to have been painted during one of Van Gogh's depressive episodes.

To give you a feel for what the different episodes of bipolar disorder are like, the following are statements representative of how people with bipolar disorder might describe their experiences during episodes of mania and depression. Read each to identify those that represent a manic episode and those that represent a depressive episode.

1. I love clubbing all night! You never get tired and everyone is your friend.
2. I'm okay some of the time, but there are days when I just stay in my bed all day. Sometimes I cry.
3. Sometimes I'm a wild boy, but my friends always tell me to watch out. I'm not sure why it's a problem.
4. It feels hopeless, like nothing will ever get better.
5. When I talk to other people with bipolar, they also say how really good it feels to be up and more like themselves.
6. When I don't take my drugs I stay awake all night binge-shopping.

Mania can take on different forms in different people. People exhibiting a manic episode are often ambivalent about their medication, can feel energetic, need very little sleep, are hypersexual, can be impulsive, and believe their normal self shows during the mania. People exhibiting a depressive episode can be lethargic, disconnected, sad, hopeless, and have feelings of worthlessness. The episode types that match the statements are A: Manic; B: Depressive; C: Manic; D: Depressive; E: Manic; F: Manic.

Variability in the Duration and Nature of Bipolar Episodes

The duration of manic and depressive episodes often varies from person to person. To see how long these episodes actually lasted, beginning in 1978, David Solomon and his colleagues (2010) followed 219 patients with bipolar disorder for up to 25 years. They regularly interviewed the patients to evaluate the types and duration of episodes they were experiencing. They found that, in these patients, the typical depressive episode lasted about three months and the average manic episode lasted about one month. Duration estimates from other studies have shown slightly shorter or longer average durations for each phase, but overall a few clear conclusions can be drawn from this literature (Tondo et al., 2017). The duration of episodes is typically months as opposed to days or weeks and manic episodes tend to be significantly shorter than depressive episodes.

An exception to the long duration of the manic and depressive episodes these studies found occurs when a patient experiences **rapid cycling**. Appearing in 22 to 35 percent of bipolar patients (Miola et al., 2023), this diagnosis requires at least four mood episodes in a 12-month period.

Although the typical presentation of bipolar disorder is to have alternating periods of depression and mania, there are situations in which other patterns are observed. First, bipolar disorder with rapid cycling is more likely to show sequential periods of the same mood state, such as a manic episode followed by a euthymic period followed by a second manic episode. Second, there are patients who experience both depression and mania on the same day or even at the same time (Solé et al., 2017). These are called **mixed episodes** and they may seem paradoxical if you view the depressive and manic phases as opposites, but the existence of mixed episodes suggests the need for a more nuanced view (Swann, 2013).

Experiencing episodes of mixed features is common among bipolar patients. In one study, two-thirds of individuals with bipolar disorder with depressive episodes had concurrent manic symptoms (Goldberg et al., 2009). Research has discovered that bipolar patients with mixed features develop the disorder earlier in life, are more likely to be diagnosed with bipolar I disorder, and have a higher number of suicide attempts than bipolar patients without mixed symptoms (Goldberg et al., 2009; Solé et al., 2017).

What do mixed states look like? To receive the mixed state specifier with a bipolar disorder diagnosis, a patient must meet the criteria for either mania, hypomania, or a depressive episode and possess several attributes of the opposite state. For example, someone fitting the criteria for a depressive episode may also have pressured speech, experience racing thoughts, and decreased drive to sleep typical of mania. Similarly, someone fitting the criteria for a manic episode in which their mood is irritable rather than euphoric may also experience anhedonia, slowing of movement, and suicidal ideation distinctive of depression. A person who has a single mixed episode is at a greater risk of having subsequent mixed episodes, though people can experience a single mixed episode and then go back to having only separate depressive and manic episodes (Kessing, 2008).

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